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Feelings of Unreality: *A Conceptual and Phenomenological Analysis of the Language of Depersonalization*

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Abstract: The paper offers a conceptual and phenomenological analysis of the language of depersonalization. The depersonalization syndrome or disorder has no known common pathogenesis and shows no characteristic behavioral manifestations. A conceptual analysis of the key terms in the subjective complaints would therefore have consequences for clinical research into the phenomenon of depersonalization.

Characteristic reports of depersonalization symptoms are arranged according to their conceptual and psychological kinship. A scheme, which aims to separate reports that reflect an underlying depersonalization experience from similar remarks that are common in everyday language but are not considered to be connected with depersonalization, is outlined.

Further, we focus on the two dominating criteria found in the literature on depersonalization, the feelings of unreality and the so-called as-if character. Two different uses of the phrase "I feel unreal" are made explicit in order to avoid conflation. The as-if element, which usually accompanies depersonalization complaints, is discussed in the light of potential epistemologic reservations, which may motivate the use of as-if locutions. Finally, we suggest how occurrences of metaphorical language may be understood when patients verbally attempt to articulate depersonalization symptoms.

Keywords: Conceptual analysis, depersonalization, derealization, metaphor, phenomenology, unreality.

Introduction

"I seem to have no personality, as if I had no background, no future and no ties at all with anyone or anything. I feel non-existent as a personality—like a vacuum."
—Anonymous patient quoted in Ackner (1954)

"There was nothing on earth. No living being was on earth. It was like a wall before me. It was so unreal."
—Anonymous patient quoted in Lindqvist (1966)

"I'm not part of anything and so nothing seems real. The part of me that is there talking is like part of a machine. I seem so unreal to myself."
—Anonymous patient quoted in Ackner (1954)

These strange descriptions are, according to the quoted authors, attempts to capture depersonalization experiences. Under this heading fall a variety of extraordinary

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experiences that are all connected with awareness of the self and the world. The listings of subjective reports that are diagnosed as symptoms of depersonalization differ, yet they seem to share one trait, the suggestion that the self, the body, or the world feels unreal.

In this paper, we focus on the dominating criteria found in the psychiatric literature on depersonalization: the feelings of unreality and the as-if character. After a methodologic introduction, a scheme is outlined that aims to separate reports that reflect an underlying depersonalization experience from similar remarks that are common in everyday language, but are not considered to be connected with depersonalization in the psychiatric sense. We then present a conceptual analysis of the phrase "it feels unreal," which suggests two different but interconnected uses. Finally, the as-if locution is discussed in the light of possible epistemologic reservations and the general tendency among subjects to use metaphorical language to describe experiential contents.

The Importance of Conceptual Analysis

New principles of classification are constantly being developed, but the symptom descriptions are not subject to the same attention. However, any proper clustering of symptoms in syndromes and any reliable inference from symptoms to underlying processes presuppose a proper description of the symptoms themselves. The more precise the descriptions of symptoms are, the better definitions of syndromes and the better theories about psychopathologic processes we will achieve. A conceptual analysis of the key terms in subjective reports is also helpful to the clinician who faces the task of classifying symptoms as belonging or not belonging to a certain syndrome or disorder. The depersonalization syndrome or disorder has no known common etiology or pathogenesis, and the condition shows no characteristic behavioral manifestations.[1] The subjective complaints are the main foundation for diagnosis. Hence, a conceptual analysis of depersonalization complaints is of particularly great importance.

To clarify our approach to the subject, we suggest a heuristic separation of the field of psychological and psychiatric investigation into three levels: the phenomenological, the descriptive, and the nosological levels. The first level represents the patient's subjective experience as experienced by him or her, the second the direct observations made by the psychiatrist (including the complaints offered to the psychiatrist by the patient), and the third is diagnostic classification. There is a slight confusion among psychiatric writers over the term *phenomenology*. Berrios (1992) describes four meanings in psychiatry. One is a usage that goes back to Karl Jaspers (1913, 1963) and before him to Husserl (1900) and refers to a methodology for describing mental states in a theoretically neutral manner. This is also what we mean by the term *phenomenology*. The *phenomenological method* is the attempt to describe our experiences directly, as they appear, separately from their origins and development, independently of causal or functional explanations. Our approach is partly phenomenological, partly conceptual. By a careful evaluation of what the patient really says and what he or she could possibly refer to by it, we hope to get closer to an underlying phenomenological reality.[2]

It should be noted that the descriptions given by patients who have depersonalization experiences are not always regarded as appropriate by the patients themselves. On the contrary, these patients are often disappointed with their attempts to describe their experiences. Hence, a full clarification of depersonalization complaints cannot be achieved by only analyzing standard linguistic meaning. Due care must also be taken to the fact that patients' reports may be colored by what they themselves have read in popular and/or psychiatric literature, or heard in previous contact with psychiatrists. Other important factors that may influence the reports are cultural and social belonging.

Symptom and Syndrome

The term *depersonalization* can be understood either as referring to an experience of a certain kind, or to a psychiatric disorder or syndrome. In most psychiatric systems it is recognized that

mild depersonalization experiences may appear without any psychopathologic background. It is also rather generally agreed that depersonalization experiences can occur as secondary symptoms in several other different psychiatric syndromes. Finally, almost all psychiatric systems allow for a syndrome or disorder labeled *depersonalization*. The condition was first described by Krishaber (1872) as *névropathie cérébro-cardiaque*; later Dugas (Dugas and Moutier 1911) gave this symptom complex the name *depersonalization*. For our study, we have taken our departure from the works of Schilder (1935), Ackner (1954), Lindqvist (1966), Sedman (1970), Lindqvist and Malmgren (1990), the DSM-IV-TR (APA, 2000), and Sierra and Berrios (2000), among others. We collected our case study material from Ackner and Lindqvist.

What is a depersonalization experience? Here is Paul Schilder's account from 1950:

"the individual feels completely changed from what he was previously. This change is present in the ego as well as in the outside world and the individual does not recognize himself as a personality. His actions appear to him as automatic. He observes his actions and behaviour from the point of view of a spectator. The outside world is foreign and new to him and not as real as before." (p. 138)

According to Ackner (1954) the leading features are said to be:

"a subjective feeling of internal and/or external change, experienced as one of strangeness or unreality; an unpleasant or even highly distressing quality to the experience; the retention of insight and lack of delusional elaboration of the experience; and an affective disturbance characterized often by the complaint of loss of affective responsiveness." (p. 845)

The DSM-IV-TR describes the essential features of Depersonalization Disorder like this:

"persistent or recurrent episodes of depersonalization characterized by a feeling of detachment or estrangement from one's self (. . .) The individual may feel like an automaton or as if he or she is living in a dream or a movie. There may be a sensation of being an outside observer of one's mental processes, one's body, or parts of one's body. Various types of sensory anesthesia, lack of affective response, and a sensation of lacking control of one's actions, including speech, are often present." (p. 530)

Recently, different rating scales have been put forward in order to capture and measure instances of depersonalization. See for instance, Simeon et al. (1998) and Sierra and Berrios (2000). As an example, in the questionnaire proposed by Sierra and Berrios, feelings of unreality are referred to in four items:

"(1) Out of the blue, I feel strange, as if I were not real or as if I were cut off from the world."

"(11) Familiar voices (including my own) sound remote and unreal."

"(13) My surroundings feel detached or unreal, as if there were a veil between me and the outside world."

"(27) I have to touch myself to make sure that I have a body or a real existence." (Sierra and Berrios 2000)

The diverging characterizations quoted here make up only a fraction of all the descriptions found in the literature. However, experiences of unreality and the nondelusional character of the state are traits all authors emphasize. Now, the term *unreal* as well as the phrase *it feels unreal,* as they appear in all these descriptions, are ambiguous and allow for significantly different interpretations. To increase the precision of diagnoses of depersonalization, a clarification of these interpretations is necessary.

A Catalogue of Unreality Feelings

When should a complaint about feelings of unreality be regarded as a description of depersonalization? On the one hand, people can express that things feel unreal, without this remark reflecting an underlying depersonalization experience and, on the other hand, a remark must not necessarily include the term *unreal* to count as a depersonalization complaint. To get an overview of the field of relevant descriptions we have arranged a collection of *unreality* complaints quoted in the literature according to their main reference target that is, what is said to be unreal (Table 1).

In category *A* belong descriptions with reference to self, personality, or I; more generally,

Table 1. Some Varieties of Feelings of Unreality

A. Reference to Abstract Aspects of the Self	B. Reference to Aspects of the Body	C. Reference to the External World	D. Existential Reflections	E. Reactions to Changes in Life
"I feel changed."	"I feel as if I don't belong to my own body."	"Familiar streets seem different."	"Sometimes a feeling of unreality comes over me. I am doing the same things every day. Is this my life? Is this the way it is supposed to be?"	"It feels unreal that I won at the lottery this week."
"It feels like I have no future and no past."	"My hands and feet feel detached as though they do not belong to me and sometimes they seem to disappear altogether."	"I feel isolated from the world as though it is not interested in me at all . . . Voices seem distant and different."		"It feels unreal that he isn't around any more."
"This is a living death, I have no self, it is gone and nothing is left."	"Even my own body looks unfamiliar to me."	"Things look dull, flat and lifeless."		
"I felt changed, lacking something I used to have, as though I had no personality and was only existing."	"The body felt as if it was not my own but lying beside myself."	"It was like a wall before me. It was so unreal."		
		"The whole universe and everything is strange."		

various nonbodily aspects of the self. Category *B* comprises descriptions that mention the body or parts of it. Under *C* fall descriptions of what is commonly known as *derealization,* that is, here the complaints of unreality are directed toward some aspect of (or the whole) external world.

It is important to note that categories *A, B,* and *C* are not mutually exclusive; not a few descriptions can be classified under two or even three of these headings. The majority of authors writing on depersonalization phenomena do not want to separate the phenomena of derealization from those of depersonalization in a categorical manner. The main reason for this is nosological, because these symptoms often occur together, but there is another reason of great philosophical importance, namely, that the self, the body, and its environment normally are experienced as one continuous whole (cf. Sims 1995). The feelings of unreality merge and the patient may not be able to say which aspect of the self or the world it is that feels unreal. A third reason not to treat the descriptions of depersonalization and derealization separately is that for all we know, the underlying feelings of unreality in the two cases are very similar and can therefore be subjected to the same analysis.

To exemplify our statement that not every mentioning of unreality feelings expresses a depersonalization complaint, we have added two extra categories. The first, *D*, comprises what we choose to call *existential reflections*. Utterances like "My life feels unreal" often means "My life is meaningless" or "My life is not what I expected it to be." Category *E* includes everyday exclamations like "Did I win? I cannot believe it, it feels so unreal!" Complaints typical of the first phase of grief also belong to category *E*.

Now, the person who gives a description of type *D* or *E* can usually elaborate it quite well, both in the sense of making it more precise and in the sense of offering a tentative explanation.

The complaints in category *D* and *E* are easily understood, both because the unreality is ascribed to things or events that we are familiar with and because most of us know from our own experience that important life events can give rise to emotional distress. There are no difficulties involved in recognizing what the death of a close relative means or to understand a person who offers an explanation like "My life feels unreal *because* it didn't turn out the way I expected it to." But if you ask somebody who suffers from feelings of unreality of type *A, B,* or *C* who just said "I seem so unreal to myself" to elaborate on that remark, he or she is less likely to be able to do so. At best, we receive more details of the same kind, that is, descriptions comprising instances of the same problematic terms.

We think that this problem of characterization has three sources. First, the phenomenology of the experience of unreality is new to the patient, it has not been verbalized before. A second reason is that most people have not experienced any situation—e.g., a philosophical discussion—where they had to assign a precise sense to existential terms like *self, world,* and *unreal.* Third, the patient usually does not feel that his or her experience has an understandable psychological cause. For all these reasons, it is not to be expected that the patient should be able to characterize his or her experience in precise terms. The description given may be the best way available to him or her for verbalizing a very strange experience.

To prevent a possible misunderstanding of our use of the scheme in Table 1, we would like to emphasize that it cannot be decided from the wording of a single utterance whether or not a person describes unreality feelings of type *A, B,* or *C.* There is naturally always a possibility that what looks (or sounds) like a typical depersonalization complaint after further investigation may turn out to be something else and vice versa. For example, our quote from DSM-IV-TR mentions "a sensation of lacking control of one's actions," which is there considered to be a description of a typical depersonalization experience (APA 2000 p. 530). However, such a description could also be expressing a statement by someone who just acted out of impulse: "I did it under the influence of very strong emotions." If so it would not, according to our scheme, fit category *A, B,* or *C.*

The Meaning of "Unreal" and "Feels Unreal"

We will now show (a) that the term *unreal* has three main uses and (b) that independently of these senses, *feels* in *feels unreal* has two main interpretations and that they are both relevant for a proper account of depersonalization. The meaning of the term *unreal* itself varies by context.

1. In one central interpretation, *unreal* can be translated into *nonexistent*. In this sense, Pegasus is unreal, as is a hallucinated bear, and an imagined friend.
2. Toy bears are of course real in the first sense, because they are not nonexistent (we are now excepting toy bears figuring in fairy tales). However, they are unreal in the sense of being artificial, fake, or made up. These terms can be applied to a real (existing) object that also pretends to be something else that, as such, is not real (does not exist). For example, the (real) Disneyworld has some pretensions to present a world of talking ducks, which certainly does not exist. Hence we can say, perhaps somewhat misleadingly, that Disneyworld is not real.
3. Let us consider another kind of example: *A real friend* usually means that a person is regarded to be a very good friend or a friend in the fullest sense ("that's what I call a friend"). *Real* here does not concern the ontological status of the friend, that is, whether he or she exists or not. Neither does it express that he or she is nonartificial, but rather that she satisfies our standards of friendship very well. Conversely, "he is no real friend" means that although a friend, he fails to satisfy some important criterion of friendship.

To sum up, in our everyday language the term *unreal* has three main uses: (1) not existing; (2) fake, made up, or artificial; and (3) not normal, atypical, unusual, or nonoptimal. As explained, sense 2 may be reducible to sense 1.

There is also an ambiguity in the term *feel,* which makes for an independent distinction among the senses of *feels unreal.* In our first interpretation (a) the meaning of *feels unreal* corresponds quite well to that of *is believed not*

to be real, where *real* is to be taken in sense 1 or 2. Such a belief may arise on the basis of various unusual experiences, which may themselves constitute a kind of unreality in sense 3 (non-normality). In the second interpretation (b), the phrase instead describes a specific kind of quasisensory experience, which is neither simply a belief in the nonexistence of something nor just any atypical feeling, and which is as a rule not anchored in another unusual experience. Let us explain the distinction by means of a few examples.

(a) In line with sense 3 of *real,* a *real self* may be understood as an *authentic , proper,* or *normal* self; that is, a self that the subject is used to and feels comfortable with. A normal sense of self does indeed imply that we feel as belonging to our bodies and do not experience ourselves as ghosts entrapped in a mechanical, robotlike body. The same goes for a distorted sense of agenthood, where a real self implies that a subject should experience himself or herself as an agent, as the owner and author of his or her actions. See Jaspers (1963), who analyzes depersonalization as a subjective loss of agenthood or as an absence of a primitive sense of mine-ness. Similar things may be said about feelings of unreality in connection with perceptions of the world. A normal experience of the world as real in this particular sense does not present the world as distinctively lifeless, cold, mechanical, or dead. It is reasonable to hypothesize that experiences of these atypical kinds may sometimes give rise to a tendency to believe that the self, or the world, does not exist or is a fake.

(b) Sometimes, however, the claim that something feels unreal cannot be analyzed as exemplified. For example, dreams can be experienced as either real or unreal despite their frequent surreal or strange contents. Dreams are often accompanied by a varying degree of a sense of reality (or unreality) that has no obvious correlation with the amount of strangeness or atypicality present in the dream. This particular feeling of unreality may be a very salient and distinct feature of a dream and is often reported by subjects after awakening.

Dreams may surely also feel unreal in sense (a). Indeed, we normally believe, at least after awakening, that what is dreamt is not real. But note that a situation in our awake life can feel dreamlike, although we do not have the slightest suspicion that the situation does not exist or is a fake. To look at a beautiful sunset or to be in a deep forest and see the sun pouring down through the treetops, may give rise to a feeling of this kind. We would perhaps like to say that the whole situation was dreamlike and we are not able to pick out one specific item in the scenario that carries the dreamlikeness.

Now it should come as no surprise if this experience of dreamlikeness was sometimes more or less tainted by its association with actual dreams, which we know are not real. If the world suddenly turns out to be more like our dreams than is usually the case, it is only human to let the shadow of a doubt fall on reality, too. In other words, even clear-cut cases of dreamlikeness (in our sense [b]) can be expected to contain a tendency to believe that what one sees does not exist. (If this is the case then the belief has to coexist with the insight that it is not reliable.) Nevertheless, one does not necessarily have to doubt the existence of a dreamlike scene even though one may be overwhelmed by its peculiar quality. We suggest that this special dream quality or particular dream atmosphere is essentially the same experience as that which we have called the feeling of unreality in sense (b).

We think that there are important differences between feelings of unreality in sense (a) and sense (b). Unreality judgments in sense (a) may be motivated by many different factors, for example, an altered sense of agenthood, a sense of loss of control, or feelings of not belonging to one's own body. In short, the existence of the self or the world is judged as unreal because they have a current strange or otherwise altered appearance. The second sense of *feels unreal* (b) is more like a distinct quasisensory or quasiperceptual state of immediate feelings of unreality manifest as an ineffable atmosphere. This particular feeling, we like to hypothesize, constitutes a phenomenological kind that is not reducible to other cases of unreality feelings. However, as mentioned, there may exist natural associations between the two kinds of feelings of unreality.

Radical experiential changes that make us judge the world or the self as nonexisting may perhaps cause unreality feelings in sense (b), and, vice versa, unreality feelings in sense (b) may make us want to doubt the existence of the world or the self.

In the theoretical literature on depersonalization, these conceptual ambiguities are sometimes hinted at implicitly but never systematically studied. For instance, Ackner (1954) explores the meaning of unreality complaints, but he solely interprets them according to the first sense.[3] He fails to recognize the phenomenological distinct, quasisensory feeling of unreality. He writes: " 'Unreality" " is a term which may be employed by patients to describe all varieties of changed experience outside their accepted range of normal variation" (p 846). Consider another example: Bird (1958) is seemingly not aware of the distinction we suggest. As far as we can see, the two senses of *feels unreal* are entangled in a confusing manner when he writes:

> "These feelings of unreality . . . consist of peculiar mental sensations, which cause a kind of mental unsteadiness or unsureness. The feeling is very much like a mental faintness and is akin to confusion, head swimming, intoxication, and delirium. Feelings of unreality, which are always unpleasant and strange, bring about a dimming or blurring both of the person's sense of identity and of his sense of reality." (p. 467)

> "The essential aspect of every form of depersonalization is . . . a change which, above all, causes uncertainty about what is real and what is not real." (p. 468)

Which meaning of *feels unreal* is the patient suffering from depersonalization using? We think it may be both. (a) The complaints of unreality feelings may be used by the patient to express a reaction to an unusual experience, a belief that what he or she feels is beyond the range of what can be accepted as normal and trustworthy. But, more importantly, they can also be used to (b) convey the feeling of unreality in a second sense, that is, a quasiperceptual phenomenal quality of the experience itself.

The As-If Character

Why does the patient choose to use words like *mechanical, dead,* or *lifeless* when he or she could not possibly have an idea of what it is like to be, for instance, dead? What is the most likely interpretation of the phrase "I feel like a robot?" Does this sentence make any sense at all? It cannot be interpreted as "I think I *am* a robot," because the depersonalized patient is aware that he or she is in fact not a robot. Neither can the intention be "I feel like I should have felt *if* I had been a robot," because he or she cannot have any knowledge of what it would be like to be a real robot.

One important aspect of depersonalization complaints is the so-called as-if character. Patients suffering from depersonalization experiences often formulate their complaints in terms of "it is *as if* . . . (the world is unreal, etc.)." The *as-if* feature is usually understood as a sign of a nondelusional state and that the patient shows intact reality testing as it is formulated in DSM-IV-TR (APA 2000). This is an important epistemologic feature exhibiting the patient's ability to recognize a fundamental difference between his or her own experience of the world and the world itself.

However, there is another dimension to the as-if character. We think that the unfamiliar and bizarre quality of these experiences makes it difficult to give a comprehensive account of them within the limits of language. It may be the case that the typical depersonalization experience cannot be captured in any literal vocabulary (except, of course, in specialized psychiatric concepts defined in terms of the typical depersonalization experiences). The only way to give an approximate account of one's subjective experiences is then to paint a picture in words. The *as-if* prefix may be a way of expressing uncertainty concerning the adequacy of the proposed descriptions, indicating that this is the best verbalization at hand.

The terms *feeling* and *feels like* may sometimes serve the same function as *as if* in a sentence, indicating that the description in question is not meant to express a belief in a real fact. Compare "I felt the world disappearing under my feet" as a reaction to a major life event. This may be one reason why the patients suffering from depersonalization often use the *feeling* lo-

cution. However, to make things even more complicated, it is equally often the case that these expressions are used in their literal sense. The phrase "I feel sad," for instance, is as a rule used to simply describe the occurrence of sadness. It may therefore be the case that in the expression "I feel like a robot," the term *feel* just indicates a description of a feeling whereas *like* is what serves as the mark of metaphorical use.

Language is limited when it comes to describing mental phenomena. Most of the terms we use to characterize mental states are borrowed from another context. Normally, we do not have to explicitly introduce such expressions as metaphors using special indicators. Why, then, are such indicators frequently used when patients speak of their depersonalization experiences? We think that the reason for this is that the metaphors chosen are introduced in novel contexts. The variety of metaphors that occur in typical depersonalization complaints have restricted preestablished metaphorical significance. For example, contrast, "I feel like an automaton" with the similar expression "I was talking automatically." The latter report is not seldom used in the context of everyday language by mentally healthy persons to express that one is talking without thinking or that one is not fully aware of one's own words. Although poets tend to be very creative in their use of words, they can rely on an established background body of metaphorical uses and on an implicit understanding that they are using language in a poetic way. This implicit shared background is unfortunately absent when patients try to verbalize the peculiar phenomenology of depersonalization. In short, the depersonalized subject has to use an innovative language to catch the content of his or her experiences, and to be understood accurately the subject also needs to stress that his or her use of language is metaphorical.

So, when a patient complains that he or she feels unreal, is the term *unreal* used in a metaphorical sense or not? To serve well as a metaphor, a term should have a literal meaning in some other context, like *robot* has. We all have at least a rough idea of what a robot is, so "I am a robot" does have a literal sense (even though we do not know which aspect of the robot-ness is intended through the metaphor). However, it is not obvious what the literal sense of "I am unreal" should be. Does for instance the description of something as unreal in the sense of *made up* involve a more literal use of *unreality* than a description of the feeling of one's own unreality? We want to leave this question open here.

Conclusions

In this paper, we have pursued some conceptual aspects of depersonalization complaints. It has been argued that the difficulties in understanding depersonalization complaints are due to (1) the vague and ambiguous philosophically oriented key terms, such as *self, world,* and *unreal*, which often accompany depersonalization reports; (2) the uniqueness and strangeness of depersonalization experiences; and (3) the fact that they often have no discernible psychological causes.

A coarse grouping of unreality feelings has been suggested by means of which some of the phenomenological complexity of the field can be illustrated. Two interpretations of the phrase "I feel unreal" have been made explicit. They have been characterized as the judgmental and the quasiperceptual senses. The fact that quasiperceptual feelings of unreality may occur independently of unreality judged or felt due to radical experiential changes suggests that they should be considered as phenomenologically autonomous kinds.

Finally, the so called as-if character of unreality complaints has been explored. We propose that apart from being a sign of a nondelusional state, this locution may also serve as an indicator expressing uncertainties regarding the adequacy of the offered descriptions. Alternatively, the as-if wording may be used to implicitly announce the occurrence of metaphors in a nonstandardized context (in contrast to, for example, the context of poetry).

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NOTES

1. For theories of the etiology of depersonalization see for instance, Sedman (1970). For a recent neurobiological study, see for example, Sierra and Berrios (1998). As an example of a cognitive approach, see Guralnik, Schmeidler, and Simeon (2000).

2. See Sims (1995), Berrios (1984, 1992), and of course Jaspers (1913, 1963), who emphasize the importance of descriptive phenomenology in psychopathology.

3. A fact that Ackner presents and that supports his analysis is that the patients who suffer from depersonalization tend to use the typical depersonalization descriptions of unreality feelings less frequently if the condition persists over a long period of time. Ackner writes, "It appears that the degree with which the patient regards an experience as unreal or strange is often in inverse relationship to his familiarity with it" (1954, 847). However, this fact does not rule out the possibility of feelings of unreality (in sense [b]), which diminishes after a period of time. The *feeling* may cease to appear after the patient has grown used to her condition, like the opposite case when fear or anxiety produce more anxiety attacks.

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